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EDITORIAL

ADDICTION SSA

Immigration status matters: the intersectional risk of immigration vulnerability and substance use disorder

Immigration status is a social determinant of health that uniquely impacts people with substance use disorders. Addiction-focused clinicians, advocates, and researchers should recognize how immigration vulnerability places additional burdens on immigrant patients with substance use disorders.

We are amid a profound global migration crisis, with a record high 280 million people worldwide living outside their country of origin [1]. Our population is increasingly on the move in response to conflict and climate change, which has innumerable health implications. Immigration status is a known social determinant of health, and immigration policies impact health and health-seeking behaviors [2,3]. Concurrently, parts of the world face an escalating addiction epidemic, with North American substance-related deaths reaching peak records [4,5]. Yet, there is little focus on the intersectional impacts of immigration vulnerability on people with substance use disorders (SUD). Facing these dualing crises, the field of addiction must evolve to better serve immigrants with SUD.

The specific challenges that immigrants with SUD face are country-specific and depend on a complex combination of documentation and healthcare access policies. The United States (US), with the highest number of immigrants worldwide and also facing an addiction epidemic, presents illustrative examples of how immigration and addiction crises intersect. We argue that immigration policies propagate the very harm and stigma that the field of addiction seeks to overcome and serve to perpetuate the structural racism and discrimination that many immigrants face [6].

Immigrants are a heterogenous group with different legal statuses that confer varying degrees of uncertainty in their daily lives. The International Organization for Migration defines migrant vulnerability as the "experience of violence, exploitation, or abuse within a migration context." [7] US immigrants have long faced the "violence of uncertainty," a term coined to describe how immigration policies promote fear, instability, and harm [8]. For immigrants with SUD, the US immigration system contributes to heightened migrant vulnerability through exposure to uncertainty, immigration-related barriers to evidence-based addiction treatment, and an increased risk of deportation for substance use.

Immigration status places undocumented patients with tenuous legal status at greatest risk for SUD complications. Immigrants afraid of legal consequences, such as deportation, are less likely to seek medical care [3]. This is magnified for those with drug or alcohol use disorders, because both are stigmatized conditions with legal consequences and limited treatment access. Moreover, fewer treatment options, let alone culturally or linguistically responsive ones, exist for undocumented immigrants who are also likely to be uninsured [9]. Many US states systematically bar undocumented individuals from obtaining government-issued photo identification [10], which can preclude their ability to obtain access to medications including buprenorphine and methadone. To mitigate these barriers to evidence-based treatment, policymakers should increase healthcare access by expanding insurance coverage for people regardless of immigration status and focus on removing bureaucratic barriers to addiction treatment access.

One study showed that undocumented immigrants at the United States-Mexico border who inject drugs are less likely to receive addiction treatment, medical care, and HIV testing when they are deported [11]. Little is understood about how detentions, deportations, and increasingly restrictive border policy may be shaping substance-related behaviors to exacerbate existing addiction and health disparities [12]. Although immigrants report alcohol and drug use and meet criteria for alcohol, cannabis, cocaine, and opioids use disorders at far lower rates than US-born individuals, nationally representative studies on substance use do not currently examine migration-related factors such as documentation status or time in the country despite importance to SUD outcomes [13]. Research should investigate these gaps while using a community-engaged approach to help mitigate the fact that immigrants are highly incentivized to under-report SUD because of fear of immigration consequences.

Even immigrants progressing along a legal pathway toward citizenship face systematic barriers if diagnosed with SUD. Under the Immigration and Nationality Act (INA) of 1952, which remains the principal governing immigration statute, being a "drug abuser or addict" (stigmatizing terms without clear diagnostic criteria) can bar immigrants from obtaining permanent residence in the United States, even for those previously granted refugee status or asylum after escaping persecution [14]. Clinicians performing the mandatory immigration medical exam for permanent residence are required to document drug use, as well as any "physical or mental disorder" with associated harmful behavior, including alcohol use disorder; either diagnosis may ultimately affect the patient's admissibility to obtain legal residency [15].

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An immigrant applying for naturalization must show that they have been a person of "good moral character" within the statutory period before application. A conviction for, or admission of drug use, during that period, can be a bar to showing "good moral character", resulting in a denial of citizenship [16]. Similarly, those found to be "habitual drunkards" (a stigmatizing term that lacks clinical accuracy) are barred from showing the requisite "good moral character" for naturalization and many forms of immigration relief [17]. Although left undefined in the immigration regulations, one circuit court concluded that a "habitual drunkard" is someone "who regularly drinks alcoholic beverages to excess" [18]. There clearly remains a considerable gap between how we, in the addiction community, diagnose SUD, and how the immigration system defines and penalizes substance use. Even a person without a SUD can suffer immigration-related punishment for substance use. For example, cannabis use, even in a state where it has been legalized, continues to carry immigration penalties that can jeopardize the ability to remain in the country [19]. The INA should be updated to match clinical and diagnostic standards and remove capricious and discretionary judgements about SUD.

Immigration judges and adjudicators need more training on addiction and its treatments. Despite no legal precedent, they tend to characterize SUD as a negative factor in applications for deportation relief, rather than viewing it as a factor indicating greater hardship to the applicant and their family if they were to be deported [20]. This means that adjudicators often make decisions based on their own personal views of addiction; judges may deny applications for relief on discretionary grounds if they view SUD as a moral failing rather than a medical condition meriting treatment.

We can look back a decade to appreciate HIV status as a parallel example of inappropriate medical exclusion in the immigration system. Under the same INA that excludes persons with SUD, people with HIV were rejected from immigrating or traveling to the United States until 2010 for fear of transmitting a communicable disease. Advocates successfully argued that there was no health justification for barring individuals with HIV and that stigmatizing policies run the risk of undermining access to medical care and worsening health outcomes.

The field of addiction has successfully leveraged research and advocacy to expand access to addiction treatment and to challenge how the US judicial legal system criminalizes substance use. There has been little focus on the consequences of the immigration legal system for immigrants who use substances. Addiction is a treatable chronic disease, and healthcare professionals have an ethical obligation to provide care to anyone who needs it, regardless of their immigration status [21]. As addiction clinicians, advocates, and researchers, we must recognize and respond when there are immigration-related barriers to treating SUD. We need to direct advocacy, research, and clinical efforts toward people living at the intersection of substance use and immigration vulnerability.

KEYWORDS

advocacy, health equity, immigration, law and policy, social determinants of health

AUTHOR CONTRIBUTIONS

Triveni DeFries: Conceptualization; formal analysis; investigation; supervision; writing. **Jacqueline Kelley:** Conceptualization; formal analysis; writing. **Marelen Martin**: Writing – review & editing. **Sarah Kimball:** Conceptualization; formal analysis; investigation; writing.

DECLARATION OF INTEREST

None.

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